NURS 1017 Unit 7 Case Study 7

64-year-old female client at a community health clinic who reports a resolved episode of right-hand weakness and jumbled speech: recognizing a TIA, modifiable and non-modifiable stroke risk factors, same-day emergency referral, FAST teaching, anticoagulant and antihypertensive teaching, and evaluation of secondary prevention.

Readiness problems: none

NURS 1017 Unit 7 Case Study 7, screen 1

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse reviews the Nurses' Notes and the Vital Signs. **Highlight** the findings that require follow-up.

Nurses' Notes | **Vital Signs**

1030: Client mentions that at about 0730 this morning, while brushing her teeth, her right hand suddenly went numb and weak and she dropped her toothbrush, and her words came out jumbled for about 15 minutes; then she felt "back to normal." She did not call anyone. Alert and oriented × 4; speech clear; smile symmetrical; hand grips equal and strong. She stopped taking her amlodipine 2 months ago because of ankle swelling and did not tell her provider. Smokes 1 pack of cigarettes per day. Has type 2 diabetes managed with metformin; capillary glucose 7.8 mmol/L. Brought her grandson to the clinic for his immunizations.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 7, screen 1 (answer key, score 6/6)

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse reviews the Nurses' Notes and the Vital Signs. **Highlight** the findings that require follow-up.

Nurses' Notes | **Vital Signs**

1030: Client mentions that at about 0730 this morning, while brushing her teeth, her right hand suddenly went numb and weak and she dropped her toothbrush, and her words came out jumbled for about 15 minutes; then she felt "back to normal." She did not call anyone. Alert and oriented × 4; speech clear; smile symmetrical; hand grips equal and strong. She stopped taking her amlodipine 2 months ago because of ankle swelling and did not tell her provider. Smokes 1 pack of cigarettes per day. Has type 2 diabetes managed with metformin; capillary glucose 7.8 mmol/L. Brought her grandson to the clinic for his immunizations.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 7, screen 2

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse at a community health clinic is caring for a 64-year-old female client who has come in for a blood pressure check.

Nurses' Notes | **Vital Signs** | **Health History**

1030: Client mentions that at about 0730 this morning, while brushing her teeth, her right hand suddenly went numb and weak and she dropped her toothbrush, and her words came out jumbled for about 15 minutes; then she felt "back to normal." She did not call anyone. Alert and oriented × 4; speech clear; smile symmetrical; hand grips equal and strong. She stopped taking her amlodipine 2 months ago because of ankle swelling and did not tell her provider. Smokes 1 pack of cigarettes per day. Has type 2 diabetes managed with metformin; capillary glucose 7.8 mmol/L. Brought her grandson to the clinic for his immunizations.

The nurse has reviewed the Health History.

➤ The nurse identifies the client's risk factors for stroke. For each risk factor, click to specify whether it is **modifiable** or **non-modifiable**.

Risk Factor	Modifiable	Non-modifiable
Hypertension	○	○
Age 64 years	○	○
Smoking 1 pack per day	○	○
Father had a stroke	○	○
Physical inactivity	○	○
Type 2 diabetes with an HbA1c of 8.6%	○	○
A previous TIA (today's episode)	○	○
Possible atrial fibrillation	○	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 7, screen 2 (answer key, score 8/8)

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse at a community health clinic is caring for a 64-year-old female client who has come in for a blood pressure check.

Nurses' Notes | **Vital Signs** | **Health History**

1030: Client mentions that at about 0730 this morning, while brushing her teeth, her right hand suddenly went numb and weak and she dropped her toothbrush, and her words came out jumbled for about 15 minutes; then she felt "back to normal." She did not call anyone. Alert and oriented × 4; speech clear; smile symmetrical; hand grips equal and strong. She stopped taking her amlodipine 2 months ago because of ankle swelling and did not tell her provider. Smokes 1 pack of cigarettes per day. Has type 2 diabetes managed with metformin; capillary glucose 7.8 mmol/L. Brought her grandson to the clinic for his immunizations.

The nurse has reviewed the Health History.

➤ The nurse identifies the client's risk factors for stroke. For each risk factor, click to specify whether it is **modifiable** or **non-modifiable**.

Risk Factor	Modifiable	Non-modifiable
Hypertension	●	○
Age 64 years	○	●
Smoking 1 pack per day	●	○
Father had a stroke	○	●
Physical inactivity	●	○
Type 2 diabetes with an HbA1c of 8.6%	●	○
A previous TIA (today's episode)	○	●
Possible atrial fibrillation	●	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 7, screen 3

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse at a community health clinic is caring for a 64-year-old female client who has come in for a blood pressure check.

Nurses' Notes | **Vital Signs** | **Health History**

1030: Client mentions that at about 0730 this morning, while brushing her teeth, her right hand suddenly went numb and weak and she dropped her toothbrush, and her words came out jumbled for about 15 minutes; then she felt "back to normal." She did not call anyone. Alert and oriented × 4; speech clear; smile symmetrical; hand grips equal and strong. She stopped taking her amlodipine 2 months ago because of ankle swelling and did not tell her provider. Smokes 1 pack of cigarettes per day. Has type 2 diabetes managed with metformin; capillary glucose 7.8 mmol/L. Brought her grandson to the clinic for his immunizations.

Complete the following sentences by choosing from the lists of options.

The client's episode is most consistent with Select... . The nurse's priority is the client's Select... , so the nurse Select... .

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 7, screen 3 (answer key, score 3/3)

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse at a community health clinic is caring for a 64-year-old female client who has come in for a blood pressure check.

Nurses' Notes | Vital Signs | Health History

1030: Client mentions that at about 0730 this morning, while brushing her teeth, her right hand suddenly went numb and weak and she dropped her toothbrush, and her words came out jumbled for about 15 minutes; then she felt "back to normal." She did not call anyone. Alert and oriented × 4; speech clear; smile symmetrical; hand grips equal and strong. She stopped taking her amlodipine 2 months ago because of ankle swelling and did not tell her provider. Smokes 1 pack of cigarettes per day. Has type 2 diabetes managed with metformin; capillary glucose 7.8 mmol/L. Brought her grandson to the clinic for his immunizations.

Complete the following sentences by choosing from the lists of options.

The client's episode is most consistent with [a transient ischemic attack]. The nurse's priority is the client's [high risk of a stroke in the next few days], so the nurse [arranges for her to be assessed in an emergency department today].

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 7, screen 4

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse at a community health clinic is caring for a 64-year-old female client who has come in for a blood pressure check.

Nurses' Notes | **Vital Signs** | **Health History**

1030: Client mentions that at about 0730 this morning, while brushing her teeth, her right hand suddenly went numb and weak and she dropped her toothbrush, and her words came out jumbled for about 15 minutes; then she felt "back to normal." She did not call anyone. Alert and oriented × 4; speech clear; smile symmetrical; hand grips equal and strong. She stopped taking her amlodipine 2 months ago because of ankle swelling and did not tell her provider. Smokes 1 pack of cigarettes per day. Has type 2 diabetes managed with metformin; capillary glucose 7.8 mmol/L. Brought her grandson to the clinic for his immunizations.

The provider agrees that the client should go to the emergency department now. Which actions should the nurse include before she leaves the clinic? **Select all that apply.**

- [ ] 1. Give her an aspirin from the clinic supply to take on the way
- [ ] 2. Teach her and her daughter the FAST signs of stroke and to call 9-1-1 if any return
- [ ] 3. Tell her she should not drive herself to the emergency department
- [ ] 4. Send a list of her medications, including the amlodipine she stopped
- [ ] 5. Ask her to come back to the clinic tomorrow if she still feels well
- [ ] 6. Phone a report to the emergency department nurse
- [ ] 7. Write down the time the symptoms started and when they resolved
- [ ] 8. Reassure her that no further testing is needed because her symptoms are gone

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 7, screen 4 (answer key, score 5/5)

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse at a community health clinic is caring for a 64-year-old female client who has come in for a blood pressure check.

Nurses' Notes | **Vital Signs** | **Health History**

1030: Client mentions that at about 0730 this morning, while brushing her teeth, her right hand suddenly went numb and weak and she dropped her toothbrush, and her words came out jumbled for about 15 minutes; then she felt "back to normal." She did not call anyone. Alert and oriented × 4; speech clear; smile symmetrical; hand grips equal and strong. She stopped taking her amlodipine 2 months ago because of ankle swelling and did not tell her provider. Smokes 1 pack of cigarettes per day. Has type 2 diabetes managed with metformin; capillary glucose 7.8 mmol/L. Brought her grandson to the clinic for his immunizations.

The provider agrees that the client should go to the emergency department now. Which actions should the nurse include before she leaves the clinic? **Select all that apply.**

- [ ] 1. Give her an aspirin from the clinic supply to take on the way
- [x] 2. Teach her and her daughter the FAST signs of stroke and to call 9-1-1 if any return
- [x] 3. Tell her she should not drive herself to the emergency department
- [x] 4. Send a list of her medications, including the amlodipine she stopped
- [ ] 5. Ask her to come back to the clinic tomorrow if she still feels well
- [x] 6. Phone a report to the emergency department nurse
- [x] 7. Write down the time the symptoms started and when they resolved
- [ ] 8. Reassure her that no further testing is needed because her symptoms are gone

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 7, screen 5

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The nurse at a community health clinic is caring for a 64-year-old female client who has come in for a blood pressure check.

Nurses' Notes | **Vital Signs** | **Health History**

1030: Client mentions that at about 0730 this morning, while brushing her teeth, her right hand suddenly went numb and weak and she dropped her toothbrush, and her words came out jumbled for about 15 minutes; then she felt "back to normal." She did not call anyone. Alert and oriented × 4; speech clear; smile symmetrical; hand grips equal and strong. She stopped taking her amlodipine 2 months ago because of ankle swelling and did not tell her provider. Smokes 1 pack of cigarettes per day. Has type 2 diabetes managed with metformin; capillary glucose 7.8 mmol/L. Brought her grandson to the clinic for his immunizations.

Day 4 1000: Client returns to the clinic after 1 night in hospital. Diagnosis: TIA; ECG showed atrial fibrillation. CT and CT angiography showed no stroke and no carotid narrowing. New medications: apixaban 5 mg PO twice daily, perindopril 4 mg PO daily (replaces amlodipine), atorvastatin 40 mg PO daily. Referral to the stroke prevention clinic in 2 weeks. She asks the nurse to "go over the new pills and what I need to change."

The nurse has reviewed the Nurses' Notes from Day 4 1000 and the Vital Signs from Day 4 1000.

➤ For each teaching point, click to specify whether the nurse should **include** it or **not include** it in the client's teaching.

Teaching Point	Include	Do not include
Take apixaban twice a day, about 12 hours apart, every day	○	○
If you miss a dose of apixaban, take two tablets at the next dose	○	○
Report black stools, blood in the urine, or bleeding gums that do not stop	○	○
Use ibuprofen for headaches	○	○
Use a soft toothbrush and an electric razor	○	○
Check with the pharmacist before taking any herbal products or supplements	○	○
Stop the perindopril on your own if you develop a dry cough	○	○
Ask about the free smoking cessation program and nicotine replacement	○	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 7, screen 5 (answer key, score 8/8)

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The nurse at a community health clinic is caring for a 64-year-old female client who has come in for a blood pressure check.

Nurses' Notes | Vital Signs | Health History

1030: Client mentions that at about 0730 this morning, while brushing her teeth, her right hand suddenly went numb and weak and she dropped her toothbrush, and her words came out jumbled for about 15 minutes; then she felt "back to normal." She did not call anyone. Alert and oriented × 4; speech clear; smile symmetrical; hand grips equal and strong. She stopped taking her amlodipine 2 months ago because of ankle swelling and did not tell her provider. Smokes 1 pack of cigarettes per day. Has type 2 diabetes managed with metformin; capillary glucose 7.8 mmol/L. Brought her grandson to the clinic for his immunizations.

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The nurse has reviewed the Nurses' Notes from Day 4 1000 and the Vital Signs from Day 4 1000.

➤ For each teaching point, click to specify whether the nurse should **include** it or **not include** it in the client's teaching.

Teaching Point	Include	Do not include
Take apixaban twice a day, about 12 hours apart, every day	●	○
If you miss a dose of apixaban, take two tablets at the next dose	○	●
Report black stools, blood in the urine, or bleeding gums that do not stop	●	○
Use ibuprofen for headaches	○	●
Use a soft toothbrush and an electric razor	●	○
Check with the pharmacist before taking any herbal products or supplements	●	○
Stop the perindopril on your own if you develop a dry cough	○	●
Ask about the free smoking cessation program and nicotine replacement	●	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 7, screen 6

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse at a community health clinic is caring for a 64-year-old female client who has come in for a blood pressure check.

Nurses' Notes | **Vital Signs** | **Health History**

1030: Client mentions that at about 0730 this morning, while brushing her teeth, her right hand suddenly went numb and weak and she dropped her toothbrush, and her words came out jumbled for about 15 minutes; then she felt "back to normal." She did not call anyone. Alert and oriented × 4; speech clear; smile symmetrical; hand grips equal and strong. She stopped taking her amlodipine 2 months ago because of ankle swelling and did not tell her provider. Smokes 1 pack of cigarettes per day. Has type 2 diabetes managed with metformin; capillary glucose 7.8 mmol/L. Brought her grandson to the clinic for his immunizations.

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Month 3 1000: Follow-up visit. Client states: "I quit smoking 6 weeks ago with the patch." Walks 30 minutes most days with a neighbour. Says she sometimes misses the evening apixaban "a few times a week because I forget." Describes FAST correctly and says she would call 9-1-1. HbA1c today 8.4%. No bleeding or bruising.

The nurse has reviewed the Nurses' Notes from Month 3 1000 and the Vital Signs from Month 3 1000.

➤ For each finding, click to specify whether it shows that an **expected outcome has been met** or that it **requires follow-up**.

Finding	Expected outcome met	Requires follow-up
BP 128/78 mm Hg	○	○
Misses the evening apixaban dose a few times a week	○	○
Quit smoking 6 weeks ago	○	○
HbA1c 8.4%	○	○
Walks 30 minutes most days	○	○
Describes the FAST signs and says she would call 9-1-1	○	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Finish attempt →

NURS 1017 Unit 7 Case Study 7, screen 6 (answer key, score 6/6)

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse at a community health clinic is caring for a 64-year-old female client who has come in for a blood pressure check.

Nurses' Notes | **Vital Signs** | **Health History**

1030: Client mentions that at about 0730 this morning, while brushing her teeth, her right hand suddenly went numb and weak and she dropped her toothbrush, and her words came out jumbled for about 15 minutes; then she felt "back to normal." She did not call anyone. Alert and oriented × 4; speech clear; smile symmetrical; hand grips equal and strong. She stopped taking her amlodipine 2 months ago because of ankle swelling and did not tell her provider. Smokes 1 pack of cigarettes per day. Has type 2 diabetes managed with metformin; capillary glucose 7.8 mmol/L. Brought her grandson to the clinic for his immunizations.

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The nurse has reviewed the Nurses' Notes from Month 3 1000 and the Vital Signs from Month 3 1000.

➤ For each finding, click to specify whether it shows that an **expected outcome has been met** or that it **requires follow-up**.

Finding	Expected outcome met	Requires follow-up
BP 128/78 mm Hg	●	○
Misses the evening apixaban dose a few times a week	○	●
Quit smoking 6 weeks ago	●	○
HbA1c 8.4%	○	●
Walks 30 minutes most days	●	○
Describes the FAST signs and says she would call 9-1-1	●	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Finish attempt →

NURS 1017 Unit 7 Case Study 8

34-year-old female client with a relapse of multiple sclerosis: focused neurological assessment, falls, urinary retention and infection risk, fatigue and heat sensitivity, nursing priorities and an interprofessional plan, mood and safety during high-dose corticosteroids, and discharge teaching for self-catheterization, energy conservation and ongoing therapy.

Readiness problems: none

NURS 1017 Unit 7 Case Study 8, screen 1

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse on a medical unit is caring for a 34-year-old female client with relapsing-remitting multiple sclerosis (MS).

Nurses' Notes | **Vital Signs**

Day 1 1400: Admitted with a relapse of MS. For 5 days her legs have felt weak and stiff, and she has double vision when looking to the left. Walks with a wide, unsteady gait and holds on to furniture; needed help to the bathroom. Says the weakness is much worse after a hot shower. Has had urinary urgency with leaking and gets up to void 4 times a night; limits her drinking to about 800 mL a day "so I don't have accidents." Bladder scan after voiding: 350 mL. Numbness of the fingertips of both hands, unchanged for 2 years. Fatigue: needs to rest after dressing. Tearful; says, "I've stopped seeing my friends. I feel like a burden to my husband." Works as a dental hygienist; has a 4-year-old son. Wears glasses for reading.

The nurse reviews the client's history and assessment. Which findings require follow-up? **Select all that apply.**

- [ ] 1. Temperature 36.9° C
- [ ] 2. Numbness of the fingertips, unchanged for 2 years
- [ ] 3. Needs help to walk to the bathroom because of weak, stiff legs and an unsteady gait
- [ ] 4. Weakness is much worse after a hot shower
- [ ] 5. Wears glasses for reading
- [ ] 6. Bladder scan of 350 mL after voiding
- [ ] 7. Limits her fluid intake to about 800 mL a day
- [ ] 8. Double vision when looking to the left
- [ ] 9. States she has stopped seeing friends and feels like a burden

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 8, screen 1 (answer key, score 6/6)

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse on a medical unit is caring for a 34-year-old female client with relapsing-remitting multiple sclerosis (MS).

Nurses' Notes | **Vital Signs**

Day 1 1400: Admitted with a relapse of MS. For 5 days her legs have felt weak and stiff, and she has double vision when looking to the left. Walks with a wide, unsteady gait and holds on to furniture; needed help to the bathroom. Says the weakness is much worse after a hot shower. Has had urinary urgency with leaking and gets up to void 4 times a night; limits her drinking to about 800 mL a day "so I don't have accidents." Bladder scan after voiding: 350 mL. Numbness of the fingertips of both hands, unchanged for 2 years. Fatigue: needs to rest after dressing. Tearful; says, "I've stopped seeing my friends. I feel like a burden to my husband." Works as a dental hygienist; has a 4-year-old son. Wears glasses for reading.

The nurse reviews the client's history and assessment. Which findings require follow-up? **Select all that apply.**

- [ ] 1. Temperature 36.9° C
- [ ] 2. Numbness of the fingertips, unchanged for 2 years
- [x] 3. Needs help to walk to the bathroom because of weak, stiff legs and an unsteady gait
- [x] 4. Weakness is much worse after a hot shower
- [ ] 5. Wears glasses for reading
- [x] 6. Bladder scan of 350 mL after voiding
- [x] 7. Limits her fluid intake to about 800 mL a day
- [x] 8. Double vision when looking to the left
- [x] 9. States she has stopped seeing friends and feels like a burden

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 8, screen 2

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse on a medical unit is caring for a 34-year-old female client with relapsing-remitting multiple sclerosis (MS).

Nurses' Notes | **Vital Signs**

Day 1 1400: Admitted with a relapse of MS. For 5 days her legs have felt weak and stiff, and she has double vision when looking to the left. Walks with a wide, unsteady gait and holds on to furniture; needed help to the bathroom. Says the weakness is much worse after a hot shower. Has had urinary urgency with leaking and gets up to void 4 times a night; limits her drinking to about 800 mL a day "so I don't have accidents." Bladder scan after voiding: 350 mL. Numbness of the fingertips of both hands, unchanged for 2 years. Fatigue: needs to rest after dressing. Tearful; says, "I've stopped seeing my friends. I feel like a burden to my husband." Works as a dental hygienist; has a 4-year-old son. Wears glasses for reading.

For each finding, click to specify the nursing problem or problems it contributes to: **risk for falls**, **risk for urinary tract infection**, or **fatigue**. Each finding may contribute to more than one problem.

Finding	Risk for falls	Risk for urinary tract infection	Fatigue
Weak, stiff legs with an unsteady gait	☐	☐	☐
Post-void residual of 350 mL	☐	☐	☐
Gets up to void 4 times a night	☐	☐	☐
Double vision	☐	☐	☐
Drinks only about 800 mL a day	☐	☐	☐
Weakness worse after a hot shower	☐	☐	☐
Needs to rest after dressing	☐	☐	☐

Note: Each column must have at least 1 response option selected.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 8, screen 2 (answer key, score 9/9)

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse on a medical unit is caring for a 34-year-old female client with relapsing-remitting multiple sclerosis (MS).

Nurses' Notes | **Vital Signs**

Day 1 1400: Admitted with a relapse of MS. For 5 days her legs have felt weak and stiff, and she has double vision when looking to the left. Walks with a wide, unsteady gait and holds on to furniture; needed help to the bathroom. Says the weakness is much worse after a hot shower. Has had urinary urgency with leaking and gets up to void 4 times a night; limits her drinking to about 800 mL a day "so I don't have accidents." Bladder scan after voiding: 350 mL. Numbness of the fingertips of both hands, unchanged for 2 years. Fatigue: needs to rest after dressing. Tearful; says, "I've stopped seeing my friends. I feel like a burden to my husband." Works as a dental hygienist; has a 4-year-old son. Wears glasses for reading.

For each finding, click to specify the nursing problem or problems it contributes to: **risk for falls**, **risk for urinary tract infection**, or **fatigue**. Each finding may contribute to more than one problem.

Finding	Risk for falls	Risk for urinary tract infection	Fatigue
Weak, stiff legs with an unsteady gait	☑	☐	☐
Post-void residual of 350 mL	☐	☑	☐
Gets up to void 4 times a night	☑	☐	☑
Double vision	☑	☐	☐
Drinks only about 800 mL a day	☐	☑	☐
Weakness worse after a hot shower	☑	☐	☑
Needs to rest after dressing	☐	☐	☑

Note: Each column must have at least 1 response option selected.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 8, screen 3

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse on a medical unit is caring for a 34-year-old female client with relapsing-remitting multiple sclerosis (MS).

Nurses' Notes | **Vital Signs**

Day 1 1400: Admitted with a relapse of MS. For 5 days her legs have felt weak and stiff, and she has double vision when looking to the left. Walks with a wide, unsteady gait and holds on to furniture; needed help to the bathroom. Says the weakness is much worse after a hot shower. Has had urinary urgency with leaking and gets up to void 4 times a night; limits her drinking to about 800 mL a day "so I don't have accidents." Bladder scan after voiding: 350 mL. Numbness of the fingertips of both hands, unchanged for 2 years. Fatigue: needs to rest after dressing. Tearful; says, "I've stopped seeing my friends. I feel like a burden to my husband." Works as a dental hygienist; has a 4-year-old son. Wears glasses for reading.

Complete the following sentences by choosing from the lists of options.

The nurse's priority problem for the client is Select... because of Select... . The nurse also addresses her Select... , which increases her risk of Select... .

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 8, screen 3 (answer key, score 4/4)

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse on a medical unit is caring for a 34-year-old female client with relapsing-remitting multiple sclerosis (MS).

Nurses' Notes | **Vital Signs**

Day 1 1400: Admitted with a relapse of MS. For 5 days her legs have felt weak and stiff, and she has double vision when looking to the left. Walks with a wide, unsteady gait and holds on to furniture; needed help to the bathroom. Says the weakness is much worse after a hot shower. Has had urinary urgency with leaking and gets up to void 4 times a night; limits her drinking to about 800 mL a day "so I don't have accidents." Bladder scan after voiding: 350 mL. Numbness of the fingertips of both hands, unchanged for 2 years. Fatigue: needs to rest after dressing. Tearful; says, "I've stopped seeing my friends. I feel like a burden to my husband." Works as a dental hygienist; has a 4-year-old son. Wears glasses for reading.

Complete the following sentences by choosing from the lists of options.

The nurse's priority problem for the client is [risk for falls] because of [her leg weakness, unsteady gait, double vision, and nighttime trips to the bathroom]. The nurse also addresses her [urinary retention and low fluid intake], which increases her risk of [a urinary tract infection that can worsen her MS symptoms].

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 8, screen 4

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse on a medical unit is caring for a 34-year-old female client with relapsing-remitting multiple sclerosis (MS).

Nurses' Notes | **Vital Signs** | **Provider Orders**

Day 1 1400: Admitted with a relapse of MS. For 5 days her legs have felt weak and stiff, and she has double vision when looking to the left. Walks with a wide, unsteady gait and holds on to furniture; needed help to the bathroom. Says the weakness is much worse after a hot shower. Has had urinary urgency with leaking and gets up to void 4 times a night; limits her drinking to about 800 mL a day "so I don't have accidents." Bladder scan after voiding: 350 mL. Numbness of the fingertips of both hands, unchanged for 2 years. Fatigue: needs to rest after dressing. Tearful; says, "I've stopped seeing my friends. I feel like a burden to my husband." Works as a dental hygienist; has a 4-year-old son. Wears glasses for reading.

The nurse has reviewed the Provider Orders.

➤ Which interventions should the nurse include in the client's plan of care? **Select all that apply.**

- [ ] 1. Plan care and activities with rest periods in between
- [ ] 2. Ask open-ended questions about her mood and coping, and involve her husband if she wishes
- [ ] 3. Apply a heating pad to her stiff legs
- [ ] 4. Encourage her to stay in bed until the relapse resolves
- [ ] 5. Insert an indwelling urinary catheter to keep her bladder empty
- [ ] 6. Keep the bed in the lowest position with the call bell and a urinal or commode within reach at night
- [ ] 7. Offer lukewarm, not hot, showers with a shower chair
- [ ] 8. Encourage a fluid intake of about 2 L a day, spread over the day and reduced in the evening

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 8, screen 4 (answer key, score 5/5)

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse on a medical unit is caring for a 34-year-old female client with relapsing-remitting multiple sclerosis (MS).

Nurses' Notes | **Vital Signs** | **Provider Orders**

Day 1 1400: Admitted with a relapse of MS. For 5 days her legs have felt weak and stiff, and she has double vision when looking to the left. Walks with a wide, unsteady gait and holds on to furniture; needed help to the bathroom. Says the weakness is much worse after a hot shower. Has had urinary urgency with leaking and gets up to void 4 times a night; limits her drinking to about 800 mL a day "so I don't have accidents." Bladder scan after voiding: 350 mL. Numbness of the fingertips of both hands, unchanged for 2 years. Fatigue: needs to rest after dressing. Tearful; says, "I've stopped seeing my friends. I feel like a burden to my husband." Works as a dental hygienist; has a 4-year-old son. Wears glasses for reading.

The nurse has reviewed the Provider Orders.

Which interventions should the nurse include in the client's plan of care? **Select all that apply.**

- [x] 1. Plan care and activities with rest periods in between
- [x] 2. Ask open-ended questions about her mood and coping, and involve her husband if she wishes
- [ ] 3. Apply a heating pad to her stiff legs
- [ ] 4. Encourage her to stay in bed until the relapse resolves
- [ ] 5. Insert an indwelling urinary catheter to keep her bladder empty
- [x] 6. Keep the bed in the lowest position with the call bell and a urinal or commode within reach at night
- [x] 7. Offer lukewarm, not hot, showers with a shower chair
- [x] 8. Encourage a fluid intake of about 2 L a day, spread over the day and reduced in the evening

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 8, screen 5

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The nurse on a medical unit is caring for a 34-year-old female client with relapsing-remitting multiple sclerosis (MS).

Nurses' Notes | **Vital Signs** | **Provider Orders** | **Laboratory Results**

Day 1 1400: Admitted with a relapse of MS. For 5 days her legs have felt weak and stiff, and she has double vision when looking to the left. Walks with a wide, unsteady gait and holds on to furniture; needed help to the bathroom. Says the weakness is much worse after a hot shower. Has had urinary urgency with leaking and gets up to void 4 times a night; limits her drinking to about 800 mL a day "so I don't have accidents." Bladder scan after voiding: 350 mL. Numbness of the fingertips of both hands, unchanged for 2 years. Fatigue: needs to rest after dressing. Tearful; says, "I've stopped seeing my friends. I feel like a burden to my husband." Works as a dental hygienist; has a 4-year-old son. Wears glasses for reading.

Day 3 0200: Client awake, sitting on the edge of the bed, crying. Says she has not slept since starting the steroid and feels "wired." States, "What's the point? I'm going to end up in a wheelchair and my son will grow up with a useless mother."

The nurse has reviewed the Nurses' Notes from Day 3 0200, the Vital Signs from Day 3 0200 and the Laboratory Results.

Which actions should the nurse take? **Select all that apply.**

- [ ] 1. Stay with her and listen to her concerns
- [ ] 2. Hold the next methylprednisolone dose until the provider sees her in the morning
- [ ] 3. Report the glucose, BP, insomnia and mood changes to the provider
- [ ] 4. Encourage her to walk the hallway alone until she feels tired
- [ ] 5. Ask her directly whether she is having thoughts of harming herself or ending her life
- [ ] 6. Tell her that steroids always cause these feelings, so she should try to ignore them
- [ ] 7. Help her back to bed and offer measures that promote sleep, such as dimming the lights and limiting interruptions

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 8, screen 5 (answer key, score 4/4)

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The nurse on a medical unit is caring for a 34-year-old female client with relapsing-remitting multiple sclerosis (MS).

Nurses' Notes | **Vital Signs** | **Provider Orders** | **Laboratory Results**

Day 1 1400: Admitted with a relapse of MS. For 5 days her legs have felt weak and stiff, and she has double vision when looking to the left. Walks with a wide, unsteady gait and holds on to furniture; needed help to the bathroom. Says the weakness is much worse after a hot shower. Has had urinary urgency with leaking and gets up to void 4 times a night; limits her drinking to about 800 mL a day "so I don't have accidents." Bladder scan after voiding: 350 mL. Numbness of the fingertips of both hands, unchanged for 2 years. Fatigue: needs to rest after dressing. Tearful; says, "I've stopped seeing my friends. I feel like a burden to my husband." Works as a dental hygienist; has a 4-year-old son. Wears glasses for reading.

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The nurse has reviewed the Nurses' Notes from Day 3 0200, the Vital Signs from Day 3 0200 and the Laboratory Results.

Which actions should the nurse take? **Select all that apply.**

- [x] 1. Stay with her and listen to her concerns
- [ ] 2. Hold the next methylprednisolone dose until the provider sees her in the morning
- [x] 3. Report the glucose, BP, insomnia and mood changes to the provider
- [ ] 4. Encourage her to walk the hallway alone until she feels tired
- [x] 5. Ask her directly whether she is having thoughts of harming herself or ending her life
- [ ] 6. Tell her that steroids always cause these feelings, so she should try to ignore them
- [x] 7. Help her back to bed and offer measures that promote sleep, such as dimming the lights and limiting interruptions

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 8, screen 6

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse on a medical unit is caring for a 34-year-old female client with relapsing-remitting multiple sclerosis (MS).

Nurses' Notes | **Vital Signs** | **Provider Orders** | **Laboratory Results**

Day 1 1400: Admitted with a relapse of MS. For 5 days her legs have felt weak and stiff, and she has double vision when looking to the left. Walks with a wide, unsteady gait and holds on to furniture; needed help to the bathroom. Says the weakness is much worse after a hot shower. Has had urinary urgency with leaking and gets up to void 4 times a night; limits her drinking to about 800 mL a day "so I don't have accidents." Bladder scan after voiding: 350 mL. Numbness of the fingertips of both hands, unchanged for 2 years. Fatigue: needs to rest after dressing. Tearful; says, "I've stopped seeing my friends. I feel like a burden to my husband." Works as a dental hygienist; has a 4-year-old son. Wears glasses for reading.

Day 3 0200: Client awake, sitting on the edge of the bed, crying. Says she has not slept since starting the steroid and feels "wired." States, "What's the point? I'm going to end up in a wheelchair and my son will grow up with a useless mother."

Day 7 1000: Leg strength improving; walks on the unit with a 4-wheeled walker and stand-by assistance. Double vision resolved. Performs clean intermittent self-catheterization every 6 hours; residuals 150–250 mL. Drinks about 2 L a day. Has started counselling through the MS clinic; depression screen shows mild symptoms. Discharge home tomorrow. The nurse reviews the discharge teaching with the client and her husband.

The nurse has reviewed the Nurses' Notes from Day 7 1000.

➤ For each statement, click to specify whether it indicates an **understanding** of the discharge teaching or a **need for further teaching**.

Statement	Understanding	Needs further teaching
Client: "I'll wash my hands and the catheter, and catheterize every 6 hours, even when I'm busy at work."	○	○
Husband: "She should stop the disease-modifying injections once she feels back to normal."	○	○
Client: "If my urine burns, smells bad, or I get a fever, I'll call the MS clinic."	○	○
Client: "I'll use my walker at home until physiotherapy says I can stop."	○	○
Client: "I'll save my errands for the hottest part of the afternoon, when I've had time to rest."	○	○
Husband: "We'll ask the occupational therapist about setting up the kitchen so she can sit while she works."	○	○
Client: "I'll keep my counselling appointments, even if I'm feeling better."	○	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Finish attempt →

NURS 1017 Unit 7 Case Study 8, screen 6 (answer key, score 7/7)

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse on a medical unit is caring for a 34-year-old female client with relapsing-remitting multiple sclerosis (MS).

Nurses' Notes | Vital Signs | Provider Orders | Laboratory Results

Day 1 1400: Admitted with a relapse of MS. For 5 days her legs have felt weak and stiff, and she has double vision when looking to the left. Walks with a wide, unsteady gait and holds on to furniture; needed help to the bathroom. Says the weakness is much worse after a hot shower. Has had urinary urgency with leaking and gets up to void 4 times a night; limits her drinking to about 800 mL a day "so I don't have accidents." Bladder scan after voiding: 350 mL. Numbness of the fingertips of both hands, unchanged for 2 years. Fatigue: needs to rest after dressing. Tearful; says, "I've stopped seeing my friends. I feel like a burden to my husband." Works as a dental hygienist; has a 4-year-old son. Wears glasses for reading.

Day 3 0200: Client awake, sitting on the edge of the bed, crying. Says she has not slept since starting the steroid and feels "wired." States, "What's the point? I'm going to end up in a wheelchair and my son will grow up with a useless mother."

Day 7 1000: Leg strength improving; walks on the unit with a 4-wheeled walker and stand-by assistance. Double vision resolved. Performs clean intermittent self-catheterization every 6 hours; residuals 150–250 mL. Drinks about 2 L a day. Has started counselling through the MS clinic; depression screen shows mild symptoms. Discharge home tomorrow. The nurse reviews the discharge teaching with the client and her husband.

The nurse has reviewed the Nurses' Notes from Day 7 1000.

For each statement, click to specify whether it indicates an **understanding** of the discharge teaching or a **need for further teaching**.

Statement	Understanding	Needs further teaching
Client: "I'll wash my hands and the catheter, and catheterize every 6 hours, even when I'm busy at work."	●	○
Husband: "She should stop the disease-modifying injections once she feels back to normal."	○	●
Client: "If my urine burns, smells bad, or I get a fever, I'll call the MS clinic."	●	○
Client: "I'll use my walker at home until physiotherapy says I can stop."	●	○
Client: "I'll save my errands for the hottest part of the afternoon, when I've had time to rest."	○	●
Husband: "We'll ask the occupational therapist about setting up the kitchen so she can sit while she works."	●	○
Client: "I'll keep my counselling appointments, even if I'm feeling better."	●	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Finish attempt →

NURS 1017 Unit 7 Case Study 9

20-year-old university student with epilepsy admitted after a breakthrough tonic-clonic seizure: seizure triggers (missed medication, sleep loss, alcohol, missed meals), describing and classifying seizures from witness accounts, seizure precautions, care during and after a seizure (bow-tie), documentation, and self-management teaching.

Readiness problems: none

NURS 1017 Unit 7 Case Study 9, screen 1

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse reviews the Nurses' Notes. **Highlight** the findings that may have contributed to the client's seizure.

Nurses' Notes

Day 1 1600: Admitted for observation after a tonic-clonic seizure in the university library this morning; brought in by ambulance. Diagnosed with epilepsy at age 15; takes levetiracetam 750 mg PO twice daily. Says she ran out of levetiracetam 3 days ago and has not had time to refill it. Has been studying for final exams, sleeping about 4 hours a night for the past week. Drank 5 beers at a friend's party last night. Skipped breakfast and lunch today. Had a coffee before going to the library. The library has ordinary overhead lighting. Exercises at the campus gym 3 times a week. Before today, her last seizure was 14 months ago. Now alert and oriented; sore tongue with a small bite on the right side; muscles ache.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 9, screen 1 (answer key, score 4/4)

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse reviews the Nurses' Notes. **Highlight** the findings that may have contributed to the client's seizure.

Nurses' Notes

Day 1 1600: Admitted for observation after a tonic-clonic seizure in the university library this morning; brought in by ambulance. Diagnosed with epilepsy at age 15; takes levetiracetam 750 mg PO twice daily. Says she ran out of levetiracetam 3 days ago and has not had time to refill it. Has been studying for final exams, sleeping about 4 hours a night for the past week. Drank 5 beers at a friend's party last night. Skipped breakfast and lunch today. Had a coffee before going to the library. The library has ordinary overhead lighting. Exercises at the campus gym 3 times a week. Before today, her last seizure was 14 months ago. Now alert and oriented; sore tongue with a small bite on the right side; muscles ache.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 9, screen 2

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse on a medical unit is caring for a 20-year-old female university student with epilepsy.

Nurses' Notes | **Vital Signs**

Day 1 1600: Admitted for observation after a tonic-clonic seizure in the university library this morning; brought in by ambulance. Diagnosed with epilepsy at age 15; takes levetiracetam 750 mg PO twice daily. Says she ran out of levetiracetam 3 days ago and has not had time to refill it. Has been studying for final exams, sleeping about 4 hours a night for the past week. Drank 5 beers at a friend's party last night. Skipped breakfast and lunch today. Had a coffee before going to the library. The library has ordinary overhead lighting. Exercises at the campus gym 3 times a week. Before today, her last seizure was 14 months ago. Now alert and oriented; sore tongue with a small bite on the right side; muscles ache.

➤ The client's roommate has witnessed several of her seizures and describes them to the nurse. For each description, click to specify the type of seizure it is most consistent with.

Description	Focal aware	Focal impaired awareness	Generalized tonic-clonic	Absence
"She gets a rising feeling in her stomach and smells burning rubber, but she can still talk to me."	○	○	○	○
"She stiffens, falls, and her arms and legs jerk; afterwards she's confused and sleeps for an hour."	○	○	○	○
"She stares, smacks her lips, and fiddles with her sleeve; she doesn't answer me and doesn't remember it later."	○	○	○	○
"When she was a child, she would stare blankly for a few seconds many times a day, then carry on as if nothing happened."	○	○	○	○
"Her right hand twitches for about a minute, and she watches it happen and tells me about it."	○	○	○	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 9, screen 2 (answer key, score 5/5)

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse on a medical unit is caring for a 20-year-old female university student with epilepsy.

Nurses' Notes | **Vital Signs**

Day 1 1600: Admitted for observation after a tonic-clonic seizure in the university library this morning; brought in by ambulance. Diagnosed with epilepsy at age 15; takes levetiracetam 750 mg PO twice daily. Says she ran out of levetiracetam 3 days ago and has not had time to refill it. Has been studying for final exams, sleeping about 4 hours a night for the past week. Drank 5 beers at a friend's party last night. Skipped breakfast and lunch today. Had a coffee before going to the library. The library has ordinary overhead lighting. Exercises at the campus gym 3 times a week. Before today, her last seizure was 14 months ago. Now alert and oriented; sore tongue with a small bite on the right side; muscles ache.

➤ The client's roommate has witnessed several of her seizures and describes them to the nurse. For each description, click to specify the type of seizure it is most consistent with.

Description	Focal aware	Focal impaired awareness	Generalized tonic-clonic	Absence
"She gets a rising feeling in her stomach and smells burning rubber, but she can still talk to me."	●	○	○	○
"She stiffens, falls, and her arms and legs jerk; afterwards she's confused and sleeps for an hour."	○	○	●	○
"She stares, smacks her lips, and fiddles with her sleeve; she doesn't answer me and doesn't remember it later."	○	●	○	○
"When she was a child, she would stare blankly for a few seconds many times a day, then carry on as if nothing happened."	○	○	○	●
"Her right hand twitches for about a minute, and she watches it happen and tells me about it."	●	○	○	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 9, screen 3

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse on a medical unit is caring for a 20-year-old female university student with epilepsy.

Nurses' Notes | **Vital Signs**

Day 1 1600: Admitted for observation after a tonic-clonic seizure in the university library this morning; brought in by ambulance. Diagnosed with epilepsy at age 15; takes levetiracetam 750 mg PO twice daily. Says she ran out of levetiracetam 3 days ago and has not had time to refill it. Has been studying for final exams, sleeping about 4 hours a night for the past week. Drank 5 beers at a friend's party last night. Skipped breakfast and lunch today. Had a coffee before going to the library. The library has ordinary overhead lighting. Exercises at the campus gym 3 times a week. Before today, her last seizure was 14 months ago. Now alert and oriented; sore tongue with a small bite on the right side; muscles ache.

Complete the following sentences by choosing from the lists of options.

While the client is in hospital, the nurse's priority problem is Select...

because Select... . Before discharge, the nurse's teaching will focus first on

Select... .

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 9, screen 3 (answer key, score 3/3)

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse on a medical unit is caring for a 20-year-old female university student with epilepsy.

Nurses' Notes | **Vital Signs**

Day 1 1600: Admitted for observation after a tonic-clonic seizure in the university library this morning; brought in by ambulance. Diagnosed with epilepsy at age 15; takes levetiracetam 750 mg PO twice daily. Says she ran out of levetiracetam 3 days ago and has not had time to refill it. Has been studying for final exams, sleeping about 4 hours a night for the past week. Drank 5 beers at a friend's party last night. Skipped breakfast and lunch today. Had a coffee before going to the library. The library has ordinary overhead lighting. Exercises at the campus gym 3 times a week. Before today, her last seizure was 14 months ago. Now alert and oriented; sore tongue with a small bite on the right side; muscles ache.

Complete the following sentences by choosing from the lists of options.

While the client is in hospital, the nurse's priority problem is [risk for injury and aspiration] because [her levetiracetam levels are low, so another seizure may occur]. Before discharge, the nurse's teaching will focus first on [taking her antiseizure medication every day without running out].

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 9, screen 4

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse on a medical unit is caring for a 20-year-old female university student with epilepsy.

Nurses' Notes | **Vital Signs** | **Provider Orders**

Day 1 1600: Admitted for observation after a tonic-clonic seizure in the university library this morning; brought in by ambulance. Diagnosed with epilepsy at age 15; takes levetiracetam 750 mg PO twice daily. Says she ran out of levetiracetam 3 days ago and has not had time to refill it. Has been studying for final exams, sleeping about 4 hours a night for the past week. Drank 5 beers at a friend's party last night. Skipped breakfast and lunch today. Had a coffee before going to the library. The library has ordinary overhead lighting. Exercises at the campus gym 3 times a week. Before today, her last seizure was 14 months ago. Now alert and oriented; sore tongue with a small bite on the right side; muscles ache.

The nurse has reviewed the Provider Orders.

➤ Which interventions should the nurse include when setting up seizure precautions for the client? **Select all that apply.**

- [ ] 1. Keep oxygen and suction equipment ready at the bedside
- [ ] 2. Tape a padded tongue blade to the head of the bed
- [ ] 3. Take her temperature with an oral glass thermometer
- [ ] 4. Have her shower with the bathroom door unlocked and a nurse nearby, rather than take a tub bath
- [ ] 5. Remove nearby objects that could injure her, such as the bedside table
- [ ] 6. Keep the saline lock patent
- [ ] 7. Apply wrist restraints while she sleeps
- [ ] 8. Keep the bed in the lowest position with the side rails up and padded

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 9, screen 4 (answer key, score 5/5)

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse on a medical unit is caring for a 20-year-old female university student with epilepsy.

Nurses' Notes | **Vital Signs** | **Provider Orders**

Day 1 1600: Admitted for observation after a tonic-clonic seizure in the university library this morning; brought in by ambulance. Diagnosed with epilepsy at age 15; takes levetiracetam 750 mg PO twice daily. Says she ran out of levetiracetam 3 days ago and has not had time to refill it. Has been studying for final exams, sleeping about 4 hours a night for the past week. Drank 5 beers at a friend's party last night. Skipped breakfast and lunch today. Had a coffee before going to the library. The library has ordinary overhead lighting. Exercises at the campus gym 3 times a week. Before today, her last seizure was 14 months ago. Now alert and oriented; sore tongue with a small bite on the right side; muscles ache.

The nurse has reviewed the Provider Orders.

➤ Which interventions should the nurse include when setting up seizure precautions for the client? **Select all that apply.**

- [x] 1. Keep oxygen and suction equipment ready at the bedside
- [ ] 2. Tape a padded tongue blade to the head of the bed
- [ ] 3. Take her temperature with an oral glass thermometer
- [x] 4. Have her shower with the bathroom door unlocked and a nurse nearby, rather than take a tub bath
- [x] 5. Remove nearby objects that could injure her, such as the bedside table
- [x] 6. Keep the saline lock patent
- [ ] 7. Apply wrist restraints while she sleeps
- [x] 8. Keep the bed in the lowest position with the side rails up and padded

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 9, screen 5

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The nurse on a medical unit is caring for a 20-year-old female university student with epilepsy.

Nurses' Notes | **Vital Signs** | **Provider Orders**

Day 1 1600: Admitted for observation after a tonic-clonic seizure in the university library this morning; brought in by ambulance. Diagnosed with epilepsy at age 15; takes levetiracetam 750 mg PO twice daily. Says she ran out of levetiracetam 3 days ago and has not had time to refill it. Has been studying for final exams, sleeping about 4 hours a night for the past week. Drank 5 beers at a friend's party last night. Skipped breakfast and lunch today. Had a coffee before going to the library. The library has ordinary overhead lighting. Exercises at the campus gym 3 times a week. Before today, her last seizure was 14 months ago. Now alert and oriented; sore tongue with a small bite on the right side; muscles ache.

Day 2 0610: Called to the room by the roommate in the next bed at 0607: client lying in bed, unresponsive, with her whole body stiff, then rhythmic jerking of both arms and legs. Saliva frothing at the mouth. Before the jerking began, she called out "I smell burning rubber."

The nurse has reviewed the Nurses' Notes from Day 2 0610.

➤ Complete the diagram by dragging from the choices below to specify what condition the client is most likely experiencing, 2 actions the nurse should take to address that condition, and 2 parameters the nurse should monitor to assess the client's progress.



Actions to Take	Potential Condition	Parameters to Monitor
Give her sips of water	A panic attack	Urine specific gravity
Note the time the seizure started and how long it lasts	Fainting (syncope)	Airway and breathing (respiratory pattern and oxygen saturation)
Hold her arms and legs still	A tonic-clonic seizure that began as a focal aware seizure (aura)	Bowel sounds
Turn her onto her side and protect her head	An absence seizure	Capillary refill in the toes
Insert an oral airway between her teeth		Level of consciousness and orientation during recovery

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 9, screen 5 (answer key, score 5/5)

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The nurse on a medical unit is caring for a 20-year-old female university student with epilepsy.

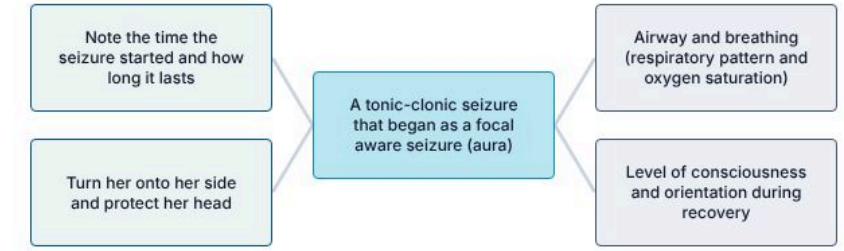
Nurses' Notes | Vital Signs | Provider Orders

Day 1 1600: Admitted for observation after a tonic-clonic seizure in the university library this morning; brought in by ambulance. Diagnosed with epilepsy at age 15; takes levetiracetam 750 mg PO twice daily. Says she ran out of levetiracetam 3 days ago and has not had time to refill it. Has been studying for final exams, sleeping about 4 hours a night for the past week. Drank 5 beers at a friend's party last night. Skipped breakfast and lunch today. Had a coffee before going to the library. The library has ordinary overhead lighting. Exercises at the campus gym 3 times a week. Before today, her last seizure was 14 months ago. Now alert and oriented; sore tongue with a small bite on the right side; muscles ache.

Day 2 0610: Called to the room by the roommate in the next bed at 0607: client lying in bed, unresponsive, with her whole body stiff, then rhythmic jerking of both arms and legs. Saliva frothing at the mouth. Before the jerking began, she called out "I smell burning rubber."

The nurse has reviewed the Nurses' Notes from Day 2 0610.

➤ Complete the diagram by dragging from the choices below to specify what condition the client is most likely experiencing, 2 actions the nurse should take to address that condition, and 2 parameters the nurse should monitor to assess the client's progress.



Actions to Take	Potential Condition	Parameters to Monitor
Give her sips of water	A panic attack	Urine specific gravity
	Fainting (syncope)	
Hold her arms and legs still		Bowel sounds
	An absence seizure	Capillary refill in the toes
Insert an oral airway between her teeth		

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 9, screen 6

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse on a medical unit is caring for a 20-year-old female university student with epilepsy.

Nurses' Notes | **Vital Signs** | **Provider Orders**

Day 1 1600: Admitted for observation after a tonic-clonic seizure in the university library this morning; brought in by ambulance. Diagnosed with epilepsy at age 15; takes levetiracetam 750 mg PO twice daily. Says she ran out of levetiracetam 3 days ago and has not had time to refill it. Has been studying for final exams, sleeping about 4 hours a night for the past week. Drank 5 beers at a friend's party last night. Skipped breakfast and lunch today. Had a coffee before going to the library. The library has ordinary overhead lighting. Exercises at the campus gym 3 times a week. Before today, her last seizure was 14 months ago. Now alert and oriented; sore tongue with a small bite on the right side; muscles ache.

Day 2 0610: Called to the room by the roommate in the next bed at 0607: client lying in bed, unresponsive, with her whole body stiff, then rhythmic jerking of both arms and legs. Saliva frothing at the mouth. Before the jerking began, she called out "I smell burning rubber."

Day 2 0614: Seizure stopped at 0609 (2 minutes). Turned to her left side; oral suction for saliva; oxygen 2 L/min by nasal cannula; SpO_2 96%. Postictal: drowsy, oriented to person only at 0630, oriented × 4 at 0715. Provider notified.

Day 3 1000: No further seizures. Pharmacist arranged a 3-month supply and automatic refills. Discharge today. The nurse reviews the teaching with the client; her roommate is present.

The nurse has reviewed the Nurses' Notes from Day 2 0614 and Day 3 1000.

For each statement, click to specify whether it indicates an **understanding** of the teaching or a **need for further teaching**.

Statement	Understanding	Needs further teaching
Client: "I'll set alarms on my phone for my levetiracetam and refill it a week before I run out."	○	○
Client: "If I miss a dose, I'll just take two at the next dose."	○	○
Client: "I won't drive until my neurologist tells me it's safe under the provincial rules."	○	○
Client: "I can stay up all night studying as long as I take my pills."	○	○
Roommate: "If she has a seizure, I'll turn her on her side, time it, and call 9-1-1 if it lasts 5 minutes."	○	○
Client: "I'll swim only with a friend who knows about my epilepsy."	○	○
Client: "I'll wear my medical alert bracelet."	○	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Finish attempt →

NURS 1017 Unit 7 Case Study 9, screen 6 (answer key, score 7/7)

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse on a medical unit is caring for a 20-year-old female university student with epilepsy.

Nurses' Notes | **Vital Signs** | **Provider Orders**

Day 1 1600: Admitted for observation after a tonic-clonic seizure in the university library this morning; brought in by ambulance. Diagnosed with epilepsy at age 15; takes levetiracetam 750 mg PO twice daily. Says she ran out of levetiracetam 3 days ago and has not had time to refill it. Has been studying for final exams, sleeping about 4 hours a night for the past week. Drank 5 beers at a friend's party last night. Skipped breakfast and lunch today. Had a coffee before going to the library. The library has ordinary overhead lighting. Exercises at the campus gym 3 times a week. Before today, her last seizure was 14 months ago. Now alert and oriented; sore tongue with a small bite on the right side; muscles ache.

Day 2 0610: Called to the room by the roommate in the next bed at 0607: client lying in bed, unresponsive, with her whole body stiff, then rhythmic jerking of both arms and legs. Saliva frothing at the mouth. Before the jerking began, she called out "I smell burning rubber."

Day 2 0614: Seizure stopped at 0609 (2 minutes). Turned to her left side; oral suction for saliva; oxygen 2 L/min by nasal cannula; SpO_2 96%. Postictal: drowsy, oriented to person only at 0630, oriented × 4 at 0715. Provider notified.

Day 3 1000: No further seizures. Pharmacist arranged a 3-month supply and automatic refills. Discharge today. The nurse reviews the teaching with the client; her roommate is present.

The nurse has reviewed the Nurses' Notes from Day 2 0614 and Day 3 1000.

➤ For each statement, click to specify whether it indicates an **understanding** of the teaching or a **need for further teaching**.

Statement	Understanding	Needs further teaching
Client: "I'll set alarms on my phone for my levetiracetam and refill it a week before I run out."	●	○
Client: "If I miss a dose, I'll just take two at the next dose."	○	●
Client: "I won't drive until my neurologist tells me it's safe under the provincial rules."	●	○
Client: "I can stay up all night studying as long as I take my pills."	○	●
Roommate: "If she has a seizure, I'll turn her on her side, time it, and call 9-1-1 if it lasts 5 minutes."	●	○
Client: "I'll swim only with a friend who knows about my epilepsy."	●	○
Client: "I'll wear my medical alert bracelet."	●	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Finish attempt →

NURS 1017 Unit 7 Case Study 10

58-year-old male client with ALS cared for at home: recognizing dysphagia, nighttime hypoventilation, weight loss and caregiver strain; linking findings to nursing problems; prioritizing breathing and swallowing; an interprofessional plan (SLP, dietitian, BiPAP, communication device, respite, advance care planning); respiratory distress managed within his advance directive; and evaluation of outcomes.

Readiness problems: none

NURS 1017 Unit 7 Case Study 10, screen 1

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The home care nurse is visiting a 58-year-old male client who was diagnosed with amyotrophic lateral sclerosis (ALS) 14 months ago.

Nurses' Notes | **Vital Signs**

Week 1 1000: Home visit. Lives with his wife in a bungalow; retired electrician. Uses a power wheelchair; hands weak, needs help with buttons and cutting food. Wife reports he coughs and clears his throat during most meals, and meals now take about an hour; he often stops eating because he is tired. Voice sounds wet and gurgly after drinking water. Weight 64 kg (68 kg two months ago). Sleeps in his recliner because he is short of breath lying flat; wakes with headaches and dozes during the day. Weak cough. Twitching visible in both forearms and the tongue. Speech slow and slurred but understandable; alert, does the crossword daily. Sensation intact. Wife says, "I haven't slept a full night in weeks; I'm afraid he'll choke when I'm asleep."

The nurse reviews the assessment. Which findings require follow-up? **Select all that apply.**

- [ ] 1. Morning headaches and daytime sleepiness
- [ ] 2. SpO_2 93% with shallow breathing at 22 breaths/min
- [ ] 3. Twitching of the forearm muscles
- [ ] 4. Sleeps in a recliner because he is short of breath lying flat
- [ ] 5. Weight loss of 4 kg in 2 months
- [ ] 6. Wife has not slept a full night in weeks
- [ ] 7. Sensation intact
- [ ] 8. Does the crossword daily
- [ ] 9. Coughing during most meals and a wet, gurgly voice after drinking

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 10, screen 1 (answer key, score 6/6)

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The home care nurse is visiting a 58-year-old male client who was diagnosed with amyotrophic lateral sclerosis (ALS) 14 months ago.

Nurses' Notes | **Vital Signs**

Week 1 1000: Home visit. Lives with his wife in a bungalow; retired electrician. Uses a power wheelchair; hands weak, needs help with buttons and cutting food. Wife reports he coughs and clears his throat during most meals, and meals now take about an hour; he often stops eating because he is tired. Voice sounds wet and gurgly after drinking water. Weight 64 kg (68 kg two months ago). Sleeps in his recliner because he is short of breath lying flat; wakes with headaches and dozes during the day. Weak cough. Twitching visible in both forearms and the tongue. Speech slow and slurred but understandable; alert, does the crossword daily. Sensation intact. Wife says, "I haven't slept a full night in weeks; I'm afraid he'll choke when I'm asleep."

The nurse reviews the assessment. Which findings require follow-up? **Select all that apply.**

- [x] 1. Morning headaches and daytime sleepiness
- [x] 2. SpO_2 93% with shallow breathing at 22 breaths/min
- [ ] 3. Twitching of the forearm muscles
- [x] 4. Sleeps in a recliner because he is short of breath lying flat
- [x] 5. Weight loss of 4 kg in 2 months
- [x] 6. Wife has not slept a full night in weeks
- [ ] 7. Sensation intact
- [ ] 8. Does the crossword daily
- [x] 9. Coughing during most meals and a wet, gurgly voice after drinking

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 10, screen 2

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The home care nurse is visiting a 58-year-old male client who was diagnosed with amyotrophic lateral sclerosis (ALS) 14 months ago.

Nurses' Notes | **Vital Signs**

Week 1 1000: Home visit. Lives with his wife in a bungalow; retired electrician. Uses a power wheelchair; hands weak, needs help with buttons and cutting food. Wife reports he coughs and clears his throat during most meals, and meals now take about an hour; he often stops eating because he is tired. Voice sounds wet and gurgly after drinking water. Weight 64 kg (68 kg two months ago). Sleeps in his recliner because he is short of breath lying flat; wakes with headaches and dozes during the day. Weak cough. Twitching visible in both forearms and the tongue. Speech slow and slurred but understandable; alert, does the crossword daily. Sensation intact. Wife says, "I haven't slept a full night in weeks; I'm afraid he'll choke when I'm asleep."

For each finding, click to specify the nursing problem or problems it relates to: **risk for aspiration**, **ineffective breathing pattern**, or **imbalanced nutrition: less than body requirements**. Each finding may relate to more than one problem.

Finding	Risk for aspiration	Ineffective breathing pattern	Imbalanced nutrition
Wet, gurgly voice after drinking	☐	☐	☐
Weight loss of 4 kg in 2 months	☐	☐	☐
Weak cough	☐	☐	☐
Short of breath lying flat	☐	☐	☐
Meals take an hour, and he stops eating because he is tired	☐	☐	☐
Morning headaches and daytime sleepiness	☐	☐	☐

Note: Each column must have at least 1 response option selected.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 10, screen 2 (answer key, score 8/8)

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The home care nurse is visiting a 58-year-old male client who was diagnosed with amyotrophic lateral sclerosis (ALS) 14 months ago.

Nurses' Notes | **Vital Signs**

Week 1 1000: Home visit. Lives with his wife in a bungalow; retired electrician. Uses a power wheelchair; hands weak, needs help with buttons and cutting food. Wife reports he coughs and clears his throat during most meals, and meals now take about an hour; he often stops eating because he is tired. Voice sounds wet and gurgly after drinking water. Weight 64 kg (68 kg two months ago). Sleeps in his recliner because he is short of breath lying flat; wakes with headaches and dozes during the day. Weak cough. Twitching visible in both forearms and the tongue. Speech slow and slurred but understandable; alert, does the crossword daily. Sensation intact. Wife says, "I haven't slept a full night in weeks; I'm afraid he'll choke when I'm asleep."

For each finding, click to specify the nursing problem or problems it relates to: **risk for aspiration**, **ineffective breathing pattern**, or **imbalanced nutrition: less than body requirements**. Each finding may relate to more than one problem.

Finding	Risk for aspiration	Ineffective breathing pattern	Imbalanced nutrition
Wet, gurgly voice after drinking	☑	☐	☐
Weight loss of 4 kg in 2 months	☐	☐	☑
Weak cough	☑	☑	☐
Short of breath lying flat	☐	☑	☐
Meals take an hour, and he stops eating because he is tired	☑	☐	☑
Morning headaches and daytime sleepiness	☐	☑	☐

Note: Each column must have at least 1 response option selected.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 10, screen 3

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The home care nurse is visiting a 58-year-old male client who was diagnosed with amyotrophic lateral sclerosis (ALS) 14 months ago.

Nurses' Notes | **Vital Signs**

Week 1 1000: Home visit. Lives with his wife in a bungalow; retired electrician. Uses a power wheelchair; hands weak, needs help with buttons and cutting food. Wife reports he coughs and clears his throat during most meals, and meals now take about an hour; he often stops eating because he is tired. Voice sounds wet and gurgly after drinking water. Weight 64 kg (68 kg two months ago). Sleeps in his recliner because he is short of breath lying flat; wakes with headaches and dozes during the day. Weak cough. Twitching visible in both forearms and the tongue. Speech slow and slurred but understandable; alert, does the crossword daily. Sensation intact. Wife says, "I haven't slept a full night in weeks; I'm afraid he'll choke when I'm asleep."

Complete the following sentences by choosing from the lists of options.

The nurse's highest priority is the client's Select... , followed by his Select... . Because his Select... are usually preserved in ALS, the nurse involves him directly in decisions about his care.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 10, screen 3 (answer key, score 3/3)

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The home care nurse is visiting a 58-year-old male client who was diagnosed with amyotrophic lateral sclerosis (ALS) 14 months ago.

Nurses' Notes | Vital Signs

Week 1 1000: Home visit. Lives with his wife in a bungalow; retired electrician. Uses a power wheelchair; hands weak, needs help with buttons and cutting food. Wife reports he coughs and clears his throat during most meals, and meals now take about an hour; he often stops eating because he is tired. Voice sounds wet and gurgly after drinking water. Weight 64 kg (68 kg two months ago). Sleeps in his recliner because he is short of breath lying flat; wakes with headaches and dozes during the day. Weak cough. Twitching visible in both forearms and the tongue. Speech slow and slurred but understandable; alert, does the crossword daily. Sensation intact. Wife says, "I haven't slept a full night in weeks; I'm afraid he'll choke when I'm asleep."

Complete the following sentences by choosing from the lists of options.

The nurse's highest priority is the client's [weakening breathing] , followed by his [risk for aspiration during meals] . Because his [thinking and sensation] are usually preserved in ALS, the nurse involves him directly in decisions about his care.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 10, screen 4

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The home care nurse is visiting a 58-year-old male client who was diagnosed with amyotrophic lateral sclerosis (ALS) 14 months ago.

Nurses' Notes | **Vital Signs**

Week 1 1000: Home visit. Lives with his wife in a bungalow; retired electrician. Uses a power wheelchair; hands weak, needs help with buttons and cutting food. Wife reports he coughs and clears his throat during most meals, and meals now take about an hour; he often stops eating because he is tired. Voice sounds wet and gurgly after drinking water. Weight 64 kg (68 kg two months ago). Sleeps in his recliner because he is short of breath lying flat; wakes with headaches and dozes during the day. Weak cough. Twitching visible in both forearms and the tongue. Speech slow and slurred but understandable; alert, does the crossword daily. Sensation intact. Wife says, "I haven't slept a full night in weeks; I'm afraid he'll choke when I'm asleep."

The nurse plans care with the client, his wife and the ALS clinic team. Which interventions should the nurse include? **Select all that apply.**

- [ ] 1. Arrange respite care and connect his wife with the ALS Society support program
- [ ] 2. Discuss with the team a trial of noninvasive ventilation (BiPAP) at night
- [ ] 3. Offer small, frequent, high-calorie meals and a dietitian referral
- [ ] 4. Have him drink thin liquids through a straw so they go down faster
- [ ] 5. Avoid talking about future care so he does not become upset
- [ ] 6. Encourage vigorous strength training to rebuild his muscles
- [ ] 7. Request a speech-language pathology swallowing assessment
- [ ] 8. Introduce a communication device now, while his speech is still understandable
- [ ] 9. Sit him fully upright for meals and for 30 minutes afterwards

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 10, screen 4 (answer key, score 6/6)

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The home care nurse is visiting a 58-year-old male client who was diagnosed with amyotrophic lateral sclerosis (ALS) 14 months ago.

Nurses' Notes | **Vital Signs**

Week 1 1000: Home visit. Lives with his wife in a bungalow; retired electrician. Uses a power wheelchair; hands weak, needs help with buttons and cutting food. Wife reports he coughs and clears his throat during most meals, and meals now take about an hour; he often stops eating because he is tired. Voice sounds wet and gurgly after drinking water. Weight 64 kg (68 kg two months ago). Sleeps in his recliner because he is short of breath lying flat; wakes with headaches and dozes during the day. Weak cough. Twitching visible in both forearms and the tongue. Speech slow and slurred but understandable; alert, does the crossword daily. Sensation intact. Wife says, "I haven't slept a full night in weeks; I'm afraid he'll choke when I'm asleep."

The nurse plans care with the client, his wife and the ALS clinic team. Which interventions should the nurse include? **Select all that apply.**

- [x] 1. Arrange respite care and connect his wife with the ALS Society support program
- [x] 2. Discuss with the team a trial of noninvasive ventilation (BiPAP) at night
- [x] 3. Offer small, frequent, high-calorie meals and a dietitian referral
- [ ] 4. Have him drink thin liquids through a straw so they go down faster
- [ ] 5. Avoid talking about future care so he does not become upset
- [ ] 6. Encourage vigorous strength training to rebuild his muscles
- [x] 7. Request a speech-language pathology swallowing assessment
- [x] 8. Introduce a communication device now, while his speech is still understandable
- [x] 9. Sit him fully upright for meals and for 30 minutes afterwards

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 10, screen 5

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The home care nurse is visiting a 58-year-old male client who was diagnosed with amyotrophic lateral sclerosis (ALS) 14 months ago.

Nurses' Notes | **Vital Signs**

Week 1 1000: Home visit. Lives with his wife in a bungalow; retired electrician. Uses a power wheelchair; hands weak, needs help with buttons and cutting food. Wife reports he coughs and clears his throat during most meals, and meals now take about an hour; he often stops eating because he is tired. Voice sounds wet and gurgly after drinking water. Weight 64 kg (68 kg two months ago). Sleeps in his recliner because he is short of breath lying flat; wakes with headaches and dozes during the day. Weak cough. Twitching visible in both forearms and the tongue. Speech slow and slurred but understandable; alert, does the crossword daily. Sensation intact. Wife says, "I haven't slept a full night in weeks; I'm afraid he'll choke when I'm asleep."

Week 10 1400: Uses BiPAP at night; gastrostomy tube placed in Week 6. Advance directive completed: he wants noninvasive ventilation and comfort care, and does not want intubation or CPR. Today the wife calls the nurse to the bedroom: client sitting slumped in his recliner, breathing fast and shallow, using his neck muscles, saliva pooling in his mouth, says in a whisper "can't breathe." Anxious. Lungs: coarse crackles on the right.

The nurse has reviewed the Nurses' Notes from Week 10 1400 and the Vital Signs from Week 10 1400.

Which actions should the nurse take? **Select all that apply.**

- [ ] 1. Apply his BiPAP mask
- [ ] 2. Have his wife call 9-1-1 and ask for intubation
- [ ] 3. Suction the saliva from his mouth with the oral suction device
- [ ] 4. Lay him flat to rest
- [ ] 5. Stay with him and explain each action in a calm voice
- [ ] 6. Give him a glass of water to loosen the secretions
- [ ] 7. Reposition him upright with his head supported
- [ ] 8. Contact the provider and the palliative care team about the change

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 10, screen 5 (answer key, score 5/5)

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The home care nurse is visiting a 58-year-old male client who was diagnosed with amyotrophic lateral sclerosis (ALS) 14 months ago.

Nurses' Notes | **Vital Signs**

Week 1 1000: Home visit. Lives with his wife in a bungalow; retired electrician. Uses a power wheelchair; hands weak, needs help with buttons and cutting food. Wife reports he coughs and clears his throat during most meals, and meals now take about an hour; he often stops eating because he is tired. Voice sounds wet and gurgly after drinking water. Weight 64 kg (68 kg two months ago). Sleeps in his recliner because he is short of breath lying flat; wakes with headaches and dozes during the day. Weak cough. Twitching visible in both forearms and the tongue. Speech slow and slurred but understandable; alert, does the crossword daily. Sensation intact. Wife says, "I haven't slept a full night in weeks; I'm afraid he'll choke when I'm asleep."

Week 10 1400: Uses BiPAP at night; gastrostomy tube placed in Week 6. Advance directive completed: he wants noninvasive ventilation and comfort care, and does not want intubation or CPR. Today the wife calls the nurse to the bedroom: client sitting slumped in his recliner, breathing fast and shallow, using his neck muscles, saliva pooling in his mouth, says in a whisper "can't breathe." Anxious. Lungs: coarse crackles on the right.

The nurse has reviewed the Nurses' Notes from Week 10 1400 and the Vital Signs from Week 10 1400.

Which actions should the nurse take? **Select all that apply.**

- [x] 1. Apply his BiPAP mask
- [ ] 2. Have his wife call 9-1-1 and ask for intubation
- [x] 3. Suction the saliva from his mouth with the oral suction device
- [ ] 4. Lay him flat to rest
- [x] 5. Stay with him and explain each action in a calm voice
- [ ] 6. Give him a glass of water to loosen the secretions
- [x] 7. Reposition him upright with his head supported
- [x] 8. Contact the provider and the palliative care team about the change

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 10, screen 6

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The home care nurse is visiting a 58-year-old male client who was diagnosed with amyotrophic lateral sclerosis (ALS) 14 months ago.

Nurses' Notes | **Vital Signs**

Week 1 1000: Home visit. Lives with his wife in a bungalow; retired electrician. Uses a power wheelchair; hands weak, needs help with buttons and cutting food. Wife reports he coughs and clears his throat during most meals, and meals now take about an hour; he often stops eating because he is tired. Voice sounds wet and gurgly after drinking water. Weight 64 kg (68 kg two months ago). Sleeps in his recliner because he is short of breath lying flat; wakes with headaches and dozes during the day. Weak cough. Twitching visible in both forearms and the tongue. Speech slow and slurred but understandable; alert, does the crossword daily. Sensation intact. Wife says, "I haven't slept a full night in weeks; I'm afraid he'll choke when I'm asleep."

Week 10 1400: Uses BiPAP at night; gastrostomy tube placed in Week 6. Advance directive completed: he wants noninvasive ventilation and comfort care, and does not want intubation or CPR. Today the wife calls the nurse to the bedroom: client sitting slumped in his recliner, breathing fast and shallow, using his neck muscles, saliva pooling in his mouth, says in a whisper "can't breathe." Anxious. Lungs: coarse crackles on the right.

Week 12 1000: Treated at home with antibiotics through the gastrostomy tube; fever resolved. Home visit: sleeps 6 hours with BiPAP and no longer wakes with headaches. Weight 64 kg, stable for 4 weeks on gastrostomy feeds. Uses an eye-gaze communication device to choose activities and talk with his grandchildren. A respite worker comes 2 afternoons a week. Wife gives the tube feeding with him lying flat "so he can nap." Red area over the sacrum that does not blanch.

The nurse has reviewed the Nurses' Notes from Week 12 1000 and the Vital Signs from Week 12 1000.

➤ For each finding, click to specify whether it shows that an **expected outcome has been met** or that it **requires follow-up**.

Finding	Expected outcome met	Requires follow-up
Sleeps 6 hours with BiPAP without morning headaches	○	○
Wife gives tube feedings while he lies flat	○	○
Weight stable for 4 weeks	○	○
Red area over the sacrum that does not blanch	○	○
Uses an eye-gaze device to make choices and talk with family	○	○
Respite worker visits 2 afternoons a week	○	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Finish attempt →

NURS 1017 Unit 7 Case Study 10, screen 6 (answer key, score 6/6)

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The home care nurse is visiting a 58-year-old male client who was diagnosed with amyotrophic lateral sclerosis (ALS) 14 months ago.

Nurses' Notes | **Vital Signs**

Week 1 1000: Home visit. Lives with his wife in a bungalow; retired electrician. Uses a power wheelchair; hands weak, needs help with buttons and cutting food. Wife reports he coughs and clears his throat during most meals, and meals now take about an hour; he often stops eating because he is tired. Voice sounds wet and gurgly after drinking water. Weight 64 kg (68 kg two months ago). Sleeps in his recliner because he is short of breath lying flat; wakes with headaches and dozes during the day. Weak cough. Twitching visible in both forearms and the tongue. Speech slow and slurred but understandable; alert, does the crossword daily. Sensation intact. Wife says, "I haven't slept a full night in weeks; I'm afraid he'll choke when I'm asleep."

Week 10 1400: Uses BiPAP at night; gastrostomy tube placed in Week 6. Advance directive completed: he wants noninvasive ventilation and comfort care, and does not want intubation or CPR. Today the wife calls the nurse to the bedroom: client sitting slumped in his recliner, breathing fast and shallow, using his neck muscles, saliva pooling in his mouth, says in a whisper "can't breathe." Anxious. Lungs: coarse crackles on the right.

Week 12 1000: Treated at home with antibiotics through the gastrostomy tube; fever resolved. Home visit: sleeps 6 hours with BiPAP and no longer wakes with headaches. Weight 64 kg, stable for 4 weeks on gastrostomy feeds. Uses an eye-gaze communication device to choose activities and talk with his grandchildren. A respite worker comes 2 afternoons a week. Wife gives the tube feeding with him lying flat "so he can nap." Red area over the sacrum that does not blanch.

The nurse has reviewed the Nurses' Notes from Week 12 1000 and the Vital Signs from Week 12 1000.

➤ For each finding, click to specify whether it shows that an **expected outcome has been met** or that it **requires follow-up**.

Finding	Expected outcome met	Requires follow-up
Sleeps 6 hours with BiPAP without morning headaches	◉	○
Wife gives tube feedings while he lies flat	○	◉
Weight stable for 4 weeks	◉	○
Red area over the sacrum that does not blanch	○	◉
Uses an eye-gaze device to make choices and talk with family	◉	○
Respite worker visits 2 afternoons a week	◉	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Finish attempt →

NURS 1017 Unit 7 Case Study 11

74-year-old female client with normal pressure hydrocephalus: recognizing the gait, continence and thinking changes, telling them apart from Alzheimer and Parkinson disease, the types of hydrocephalus, falls and continence priorities, care after a ventriculoperitoneal shunt, recognizing shunt malfunction at home after an MRI, and teaching about a programmable valve.

Readiness problems: none

NURS 1017 Unit 7 Case Study 11, screen 1

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse reviews the Nurses' Notes. **Highlight** the findings that require follow-up.

Nurses' Notes

Day 1 0930: Daughter reports that over the past 8 months her mother has started walking with short, shuffling steps, feet wide apart, "as if her feet are stuck to the floor", and has fallen twice at home. She has urgency and has wet herself several times; now wears pads. She has become slower to answer and less interested in her usual activities, and forgets appointments. Client is alert and oriented × 3. Wears hearing aids in both ears. Had cataract surgery 3 years ago. No tremor at rest. Reports no headache. Timed Up and Go test: 28 seconds (more than 12 seconds indicates a high fall risk). Lives alone in a two-storey house; daughter visits daily.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 11, screen 1 (answer key, score 5/5)

Practice NCLEX RN Test

Question 1 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 1 of 6

The nurse reviews the Nurses' Notes. **Highlight** the findings that require follow-up.

Nurses' Notes

Day 1 0930: Daughter reports that over the past 8 months her mother has started walking with short, shuffling steps, feet wide apart, "as if her feet are stuck to the floor", and has fallen twice at home. She has urgency and has wet herself several times; now wears pads. She has become slower to answer and less interested in her usual activities, and forgets appointments. Client is alert and oriented × 3. Wears hearing aids in both ears. Had cataract surgery 3 years ago. No tremor at rest. Reports no headache. Timed Up and Go test: 28 seconds (more than 12 seconds indicates a high fall risk). Lives alone in a two-storey house; daughter visits daily.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 11, screen 2

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse in a neurology clinic is caring for a 74-year-old female client who is accompanied by her daughter.

Nurses' Notes | **Vital Signs** | **Diagnostic Results**

Day 1 0930: Daughter reports that over the past 8 months her mother has started walking with short, shuffling steps, feet wide apart, "as if her feet are stuck to the floor", and has fallen twice at home. She has urgency and has wet herself several times; now wears pads. She has become slower to answer and less interested in her usual activities, and forgets appointments. Client is alert and oriented × 3. Wears hearing aids in both ears. Had cataract surgery 3 years ago. No tremor at rest. Reports no headache. Timed Up and Go test: 28 seconds (more than 12 seconds indicates a high fall risk). Lives alone in a two-storey house; daughter visits daily.

The nurse has reviewed the Diagnostic Results.

➤ The daughter asks whether her mother has dementia or Parkinson disease. For each finding, click to specify whether it is most consistent with **normal pressure hydrocephalus**, **Alzheimer disease**, or **Parkinson disease**.

Finding	Normal pressure hydrocephalus	Alzheimer disease	Parkinson disease
Walking improves after some CSF is removed by lumbar puncture	○	○	○
Memory loss is the first problem; walking stays normal until late in the disease	○	○	○
Resting tremor, rigidity, and slow movement	○	○	○
Wide-based, "magnetic" gait with urinary incontinence early in the illness	○	○	○
Enlarged ventricles with no blockage of CSF flow	○	○	○
Gradual loss of recent memory, getting lost in familiar places, and repeating questions	○	○	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 11, screen 2 (answer key, score 6/6)

Practice NCLEX RN Test

Question 2 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 2 of 6

The nurse in a neurology clinic is caring for a 74-year-old female client who is accompanied by her daughter.

Nurses' Notes | **Vital Signs** | **Diagnostic Results**

Day 1 0930: Daughter reports that over the past 8 months her mother has started walking with short, shuffling steps, feet wide apart, "as if her feet are stuck to the floor", and has fallen twice at home. She has urgency and has wet herself several times; now wears pads. She has become slower to answer and less interested in her usual activities, and forgets appointments. Client is alert and oriented × 3. Wears hearing aids in both ears. Had cataract surgery 3 years ago. No tremor at rest. Reports no headache. Timed Up and Go test: 28 seconds (more than 12 seconds indicates a high fall risk). Lives alone in a two-storey house; daughter visits daily.

The nurse has reviewed the Diagnostic Results.

➤ The daughter asks whether her mother has dementia or Parkinson disease. For each finding, click to specify whether it is most consistent with **normal pressure hydrocephalus**, **Alzheimer disease**, or **Parkinson disease**.

Finding	Normal pressure hydrocephalus	Alzheimer disease	Parkinson disease
Walking improves after some CSF is removed by lumbar puncture	●	○	○
Memory loss is the first problem; walking stays normal until late in the disease	○	●	○
Resting tremor, rigidity, and slow movement	○	○	●
Wide-based, "magnetic" gait with urinary incontinence early in the illness	●	○	○
Enlarged ventricles with no blockage of CSF flow	●	○	○
Gradual loss of recent memory, getting lost in familiar places, and repeating questions	○	●	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 11, screen 3

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse in a neurology clinic is caring for a 74-year-old female client who is accompanied by her daughter.

Nurses' Notes | **Vital Signs** | **Diagnostic Results**

Day 1 0930: Daughter reports that over the past 8 months her mother has started walking with short, shuffling steps, feet wide apart, "as if her feet are stuck to the floor", and has fallen twice at home. She has urgency and has wet herself several times; now wears pads. She has become slower to answer and less interested in her usual activities, and forgets appointments. Client is alert and oriented × 3. Wears hearing aids in both ears. Had cataract surgery 3 years ago. No tremor at rest. Reports no headache. Timed Up and Go test: 28 seconds (more than 12 seconds indicates a high fall risk). Lives alone in a two-storey house; daughter visits daily.

Complete the following sentences by choosing from the lists of options.

Until her shunt surgery, the nurse's priority problem for the client is Select... , because she Select... . The nurse also addresses Select... , which can increase her risk of falling at night.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 11, screen 3 (answer key, score 3/3)

Practice NCLEX RN Test

Question 3 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 3 of 6

The nurse in a neurology clinic is caring for a 74-year-old female client who is accompanied by her daughter.

Nurses' Notes | **Vital Signs** | **Diagnostic Results**

Day 1 0930: Daughter reports that over the past 8 months her mother has started walking with short, shuffling steps, feet wide apart, "as if her feet are stuck to the floor", and has fallen twice at home. She has urgency and has wet herself several times; now wears pads. She has become slower to answer and less interested in her usual activities, and forgets appointments. Client is alert and oriented × 3. Wears hearing aids in both ears. Had cataract surgery 3 years ago. No tremor at rest. Reports no headache. Timed Up and Go test: 28 seconds (more than 12 seconds indicates a high fall risk). Lives alone in a two-storey house; daughter visits daily.

Complete the following sentences by choosing from the lists of options.

Until her shunt surgery, the nurse's priority problem for the client is

risk for falls , because she

has a magnetic gait, has fallen twice, and lives alone on two floors . The nurse also addresses her urinary urgency , which can increase her risk of falling at night.

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 11, screen 4

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse in a neurology clinic is caring for a 74-year-old female client who is accompanied by her daughter.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Provider Orders**

Day 1 0930: Daughter reports that over the past 8 months her mother has started walking with short, shuffling steps, feet wide apart, "as if her feet are stuck to the floor", and has fallen twice at home. She has urgency and has wet herself several times; now wears pads. She has become slower to answer and less interested in her usual activities, and forgets appointments. Client is alert and oriented × 3. Wears hearing aids in both ears. Had cataract surgery 3 years ago. No tremor at rest. Reports no headache. Timed Up and Go test: 28 seconds (more than 12 seconds indicates a high fall risk). Lives alone in a two-storey house; daughter visits daily.

Day 15 1500: Admitted to the neurosurgical unit after insertion of a right ventriculoperitoneal (VP) shunt with a programmable valve behind the right ear. Small incisions on the right side of the head and on the abdomen. Alert and oriented × 3; pupils equal and reactive; moves all limbs equally.

The nurse has reviewed the Nurses' Notes from Day 15 1500 and the Provider Orders.

➤ Which interventions should the nurse include in the client's plan of care after shunt insertion? **Select all that apply.**

- [ ] 1. Press the shunt valve behind her ear every hour to keep it working
- [ ] 2. Assess the abdomen for pain, distension and bowel sounds
- [ ] 3. Have her lie on her right side so the valve stays warm
- [ ] 4. Walk with her using a gait belt, and offer the toilet every 2 hours
- [ ] 5. Restrict her fluid intake to reduce CSF production
- [ ] 6. Compare each neurological check with her baseline before surgery
- [ ] 7. Inspect the head and abdominal incisions and the skin along the shunt tract for redness, swelling or drainage
- [ ] 8. Include her daughter in teaching about the shunt

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 11, screen 4 (answer key, score 5/5)

Practice NCLEX RN Test

Question 4 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 4 of 6

The nurse in a neurology clinic is caring for a 74-year-old female client who is accompanied by her daughter.

Nurses' Notes | Vital Signs | Diagnostic Results | Provider Orders

Day 1 0930: Daughter reports that over the past 8 months her mother has started walking with short, shuffling steps, feet wide apart, "as if her feet are stuck to the floor", and has fallen twice at home. She has urgency and has wet herself several times; now wears pads. She has become slower to answer and less interested in her usual activities, and forgets appointments. Client is alert and oriented × 3. Wears hearing aids in both ears. Had cataract surgery 3 years ago. No tremor at rest. Reports no headache. Timed Up and Go test: 28 seconds (more than 12 seconds indicates a high fall risk). Lives alone in a two-storey house; daughter visits daily.

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The nurse has reviewed the Nurses' Notes from Day 15 1500 and the Provider Orders.

➤ Which interventions should the nurse include in the client's plan of care after shunt insertion? **Select all that apply.**

- [ ] 1. Press the shunt valve behind her ear every hour to keep it working
- [x] 2. Assess the abdomen for pain, distension and bowel sounds
- [ ] 3. Have her lie on her right side so the valve stays warm
- [x] 4. Walk with her using a gait belt, and offer the toilet every 2 hours
- [ ] 5. Restrict her fluid intake to reduce CSF production
- [x] 6. Compare each neurological check with her baseline before surgery
- [x] 7. Inspect the head and abdominal incisions and the skin along the shunt tract for redness, swelling or drainage
- [x] 8. Include her daughter in teaching about the shunt

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 11, screen 5

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The nurse in a neurology clinic is caring for a 74-year-old female client who is accompanied by her daughter.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Provider Orders**

Day 1 0930: Daughter reports that over the past 8 months her mother has started walking with short, shuffling steps, feet wide apart, "as if her feet are stuck to the floor", and has fallen twice at home. She has urgency and has wet herself several times; now wears pads. She has become slower to answer and less interested in her usual activities, and forgets appointments. Client is alert and oriented × 3. Wears hearing aids in both ears. Had cataract surgery 3 years ago. No tremor at rest. Reports no headache. Timed Up and Go test: 28 seconds (more than 12 seconds indicates a high fall risk). Lives alone in a two-storey house; daughter visits daily.

Day 15 1500: Admitted to the neurosurgical unit after insertion of a right ventriculoperitoneal (VP) shunt with a programmable valve behind the right ear. Small incisions on the right side of the head and on the abdomen. Alert and oriented × 3; pupils equal and reactive; moves all limbs equally.

Week 6 1100: Home care visit. Daughter says her mother walked much better for the first 4 weeks after surgery, but over the last 3 days she has been sleepy, has vomited twice this morning, has a headache, and is shuffling again and incontinent. Today she is drowsy but rousable and oriented to person only. Pupils equal and reactive. Incisions healed, no redness. Two days ago she had an MRI of her knee at the hospital; daughter is not sure whether the shunt valve was checked afterwards.

The nurse has reviewed the Nurses' Notes from Week 6 1100 and the Vital Signs from Week 6 1100.

Which actions should the nurse take? **Select all that apply.**

- [ ] 1. Give her a large meal to settle her stomach before she leaves
- [ ] 2. Perform and document a full neurological assessment
- [ ] 3. Tell the daughter to watch her mother and call if she is not better in a few days
- [ ] 4. Tell the emergency team about the recent MRI and the programmable valve
- [ ] 5. Keep her lying down with the head of the bed raised and stay with her until transport arrives
- [ ] 6. Contact the neurosurgery team now and arrange urgent transport to the emergency department
- [ ] 7. Press the shunt valve several times to restart the drainage

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 11, screen 5 (answer key, score 4/4)

Practice NCLEX RN Test

Question 5 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 5 of 6

The nurse in a neurology clinic is caring for a 74-year-old female client who is accompanied by her daughter.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Provider Orders**

Day 1 0930: Daughter reports that over the past 8 months her mother has started walking with short, shuffling steps, feet wide apart, "as if her feet are stuck to the floor", and has fallen twice at home. She has urgency and has wet herself several times; now wears pads. She has become slower to answer and less interested in her usual activities, and forgets appointments. Client is alert and oriented × 3. Wears hearing aids in both ears. Had cataract surgery 3 years ago. No tremor at rest. Reports no headache. Timed Up and Go test: 28 seconds (more than 12 seconds indicates a high fall risk). Lives alone in a two-storey house; daughter visits daily.

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The nurse has reviewed the Nurses' Notes from Week 6 1100 and the Vital Signs from Week 6 1100.

Which actions should the nurse take? **Select all that apply.**

- [ ] 1. Give her a large meal to settle her stomach before she leaves
- [x] 2. Perform and document a full neurological assessment
- [ ] 3. Tell the daughter to watch her mother and call if she is not better in a few days
- [x] 4. Tell the emergency team about the recent MRI and the programmable valve
- [x] 5. Keep her lying down with the head of the bed raised and stay with her until transport arrives
- [x] 6. Contact the neurosurgery team now and arrange urgent transport to the emergency department
- [ ] 7. Press the shunt valve several times to restart the drainage

Submit

← Quit

← Previous

Next →

NURS 1017 Unit 7 Case Study 11, screen 6

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse in a neurology clinic is caring for a 74-year-old female client who is accompanied by her daughter.

Nurses' Notes | **Vital Signs** | **Diagnostic Results** | **Provider Orders**

Day 1 0930: Daughter reports that over the past 8 months her mother has started walking with short, shuffling steps, feet wide apart, "as if her feet are stuck to the floor", and has fallen twice at home. She has urgency and has wet herself several times; now wears pads. She has become slower to answer and less interested in her usual activities, and forgets appointments. Client is alert and oriented × 3. Wears hearing aids in both ears. Had cataract surgery 3 years ago. No tremor at rest. Reports no headache. Timed Up and Go test: 28 seconds (more than 12 seconds indicates a high fall risk). Lives alone in a two-storey house; daughter visits daily.

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Week 6 1700: In the emergency department, the valve setting was found to have changed after the MRI; it was reset by the neurosurgeon. Drowsiness, headache and vomiting resolved overnight.

Month 3 1000: Clinic follow-up. Timed Up and Go 14 seconds; no falls since surgery. Continent with a toileting schedule. Daughter says her mother is "more like herself" and has started her book club again. The nurse reviews the teaching with the client and her daughter.

The nurse has reviewed the Nurses' Notes from Week 6 1700 and Month 3 1000.

➤ For each statement, click to specify whether it indicates an **understanding** of the teaching or a **need for further teaching**.

Statement	Understanding	Needs further teaching
Daughter: "Before any MRI, we'll tell the staff she has a programmable shunt so the setting can be checked afterwards."	○	○
Client: "Now that I have the shunt, I don't need my walker or the grab bars any more."	○	○
Daughter: "If she gets sleepy, has headaches or vomiting, or her walking gets worse again, we'll call neurosurgery right away."	○	○
Daughter: "Redness or fluid along the tubing, a fever, or pain in her belly needs to be reported."	○	○
Client: "The shunt is permanent, so I won't need any more check-ups."	○	○
Client: "I'll carry the card that says what kind of shunt valve I have."	○	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

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NURS 1017 Unit 7 Case Study 11, screen 6 (answer key, score 6/6)

Practice NCLEX RN Test

Question 6 of 6

Calculator

Question Navigator

NOT COMPLETE

Case Study Screen 6 of 6

The nurse in a neurology clinic is caring for a 74-year-old female client who is accompanied by her daughter.

Nurses' Notes | Vital Signs | Diagnostic Results | Provider Orders

Day 1 0930: Daughter reports that over the past 8 months her mother has started walking with short, shuffling steps, feet wide apart, "as if her feet are stuck to the floor", and has fallen twice at home. She has urgency and has wet herself several times; now wears pads. She has become slower to answer and less interested in her usual activities, and forgets appointments. Client is alert and oriented × 3. Wears hearing aids in both ears. Had cataract surgery 3 years ago. No tremor at rest. Reports no headache. Timed Up and Go test: 28 seconds (more than 12 seconds indicates a high fall risk). Lives alone in a two-storey house; daughter visits daily.

Day 15 1500: Admitted to the neurosurgical unit after insertion of a right ventriculoperitoneal (VP) shunt with a programmable valve behind the right ear. Small incisions on the right side of the head and on the abdomen. Alert and oriented × 3; pupils equal and reactive; moves all limbs equally.

Week 6 1100: Home care visit. Daughter says her mother walked much better for the first 4 weeks after surgery, but over the last 3 days she has been sleepy, has vomited twice this morning, has a headache, and is shuffling again and incontinent. Today she is drowsy but rousable and oriented to person only. Pupils equal and reactive. Incisions healed, no redness. Two days ago she had an MRI of her knee at the hospital; daughter is not sure whether the shunt valve was checked afterwards.

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The nurse has reviewed the Nurses' Notes from Week 6 1700 and Month 3 1000.

➤ For each statement, click to specify whether it indicates an **understanding** of the teaching or a **need for further teaching**.

Statement	Understanding	Needs further teaching
Daughter: "Before any MRI, we'll tell the staff she has a programmable shunt so the setting can be checked afterwards."	●	○
Client: "Now that I have the shunt, I don't need my walker or the grab bars any more."	○	●
Daughter: "If she gets sleepy, has headaches or vomiting, or her walking gets worse again, we'll call neurosurgery right away."	●	○
Daughter: "Redness or fluid along the tubing, a fever, or pain in her belly needs to be reported."	●	○
Client: "The shunt is permanent, so I won't need any more check-ups."	○	●
Client: "I'll carry the card that says what kind of shunt valve I have."	●	○

Note: Each row must have only 1 response option selected.

Submit

← Quit

← Previous

Finish attempt →